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BRIEF RESEARCH REPORT

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“Psychic Surgery”: Close Observation of a Popular Healing Practice

Philippine “psychic surgeons” are reputed to “enter” the body barehandedly, remove blood, tissue, and/or organs, then “close” the body, leaving no wound. In this article I report on a relatively controlled demonstration of psychic surgery held at Oakland University in September 1986. Observers included a professional magician and a pathologist. Analysis of videotapes made during the proceedings shows how a persuasive sleight-of-hand performance was achieved. Furthermore, none of the tissue or blood specimens supposedly removed during the procedure matched those of subjects. I conclude by discussing some of the forces that have contributed to the popularization of these healers.

Acceptable parapsychological research is commonly equated with carefully controlled experiments and statistical procedures for investigating such phenomena as the ability to control the fall of a coin or die (Roll, Beloff, and White 1983), “seeing” which Zener card has been turned up (Hansen 1990), or “controlling” the output of a random-number generator (Singer and Ankenbrandt 1982). Although such experiments rarely exhibit the degree of control or the mandatory replication that characterizes experiments in physics or chemistry, their form has helped create an intellectual climate of belief in “psi” phenomena. A striking example is Filipino “psychic surgery,” the apparent entry into a patient’s body with the bare hand and the extraction of a pathological agent in the form of objects like hair, seeds, dry palm leaves, or, more recently, blood and bodily tissue.

I suggest that anthropologists can study the phenomenon of psychic surgery from two different perspectives: (1) as cultural behavior and (2) as a biophysical phenomenon in and of itself. The former study can be accomplished by participant observation of the classical kind. The latter requires, in addition, specialist observers, such as magicians, audiovisual technicians, and pathologists. Furthermore, since the laws of physics and biology apply equally in Philippine barrios and air-conditioned laboratories and since “psychic surgery” purports to be a specific physical and biological phenomenon, it can be studied outside either its orig-

inal or transplanted cultural context (i.e., in those places, from Singapore to Sedona, Arizona, where Philippine healers now travel to perform psychic surgery). Wherever the procedure is performed, the material behavior can be responsibly described by ascertaining the answers to the following questions. Is the body penetrated with bare hands? Are blood and tissue removed? Is the "opening" closed without a scar?

To answer these and other biophysical questions, I arranged a demonstration of "noninvasive psychic surgery" in cooperation with the Department of Physics at Oakland University in 1986. The question of healing was specifically not addressed, since almost every type of disease may respond to placebo treatment, including the common cold, hypertension, or multiple sclerosis (Netter 1977). Consequently the demonstration was planned to focus on the alleged phenomenon itself—the act of opening the body barehandedly and removing organic material, including blood and tissue, which can be analyzed for pathology. The demonstration involved observation and specific audiovisual research methods (Sorenson and Gajdusek 1963), but it was also conducted with an awareness of the historical, political, and cultural contexts of psychic surgery and its practitioners.

The psychic surgeon in this demonstration was the Reverend Philip S. Malicdan, founder of the "Cultural Minorities Spiritual Fraternization Church of the Philippines" (Government Permit No. 44125), Baguio City. Malicdan identifies himself as an Igorot or "Mountain people," educated at the Mission School (St. Mary's) in Sagada, Bontoc, the University of the Philippines at Baguio, and the Lutheran Seminary at Baguio. He claims to have gained his power to open the body barehandedly while on a yearly fast in a cave, where he heard a voice say, "Go in peace with joy, and help the ill, and I will help you in all you do." He has established healing centers in Singapore, Australia, California, Korea, and the Netherlands, and he claims recently to have treated "Miss Austria and Miss Bolivia 1980 and her mother" (Malicdan 1986, personal communication).¹

W. H. Belk, of the Belk Foundation and 350 Belk department stores, brought Malicdan and his wife Frances, who acts as his assistant, to his home in Charlotte, North Carolina, in August 1986 for a two-month visit. Belk's plan was to demonstrate the imminence of the Biblical "end time" through the "miracles" of psychic surgery. During their visit the Malicdans treated more than 400 patients in Charlotte. On the basis of a small anonymous grant, and with the cooperation of Belk and the Department of Physics, Oakland University, I was able to set up a demonstration by Malicdan on September 24, 1986, of allegedly noninvasive psychic surgery under relatively controlled conditions and with recording instrumentation.

The demonstration was held in the Kettering Magnetics Laboratory at Oakland University in Rochester, Michigan. Malicdan "operated" on eight subjects, men and women who were chosen both because they had an interest in the phenomenon of psychic surgery and because they had a medically diagnosed condition (e.g., varicose veins, hemorrhoids). Because Malicdan asked to have the procedure stopped at various points to allow him to rest, the entire demonstration took five hours, during which time he removed material from five of the eight subjects.

The research protocol was designed to produce one of two possible results: (1) discovery of sleight-of-hand or (2) production of anomalous phenomena

which would then call for additional scientific research. While preparing the protocol, I consulted by telephone with the magician, James Randi, about using a magician in the demonstration. Randi advised a sequential approach, wherein the psychic surgeon would first be allowed to carry out his usual procedure with the magician closely observing; after the first demonstration the magician would set terms for any further enactments so as to catch the psychic surgeon “in the act.” Randi concurred in my selection of Max Maven as the magician-observer. Maven was chosen on the recommendation of Charles Reynolds of Illusion Associates in New York City, a consultant to theatrical and television enterprises that use magical effects. In a letter accompanying his final report on the Malicdan demonstration, Maven wrote that he understood his role to be “that of [an] informed observer; we had specifically agreed that I would not ‘grandstand’ ” (Maven 1987, personal communication).

As the technical consultant and audiovisual coordinator, I selected Tom Peterson of Cleveland, Ohio, who was technical advisor and consultant to the Kent State University parapsychology demonstrations and tests, held in 1975–76 under the sponsorship of Glenn Olds, then the university’s president. Peterson also acts as consultant to police and fire departments in the use of high-speed photography and explosives and has served as an expert witness on the interpretation of surveillance film for the FBI, General Electric, and other clients.

The pathologist who collected all materials “removed” or “materialized” from subjects was Alexander S. Ullmann, M.D., Chief Pathologist, Crittenton Hospital, Rochester, Michigan. All specimens collected were preserved under sterile conditions, and Ullman analyzed all such materials in his laboratory in comparison with samples of the subjects’ blood. He also sent suitable samples to the Michigan State Police Crime Laboratory for testing that was beyond the capacity of the hospital laboratory.

The Observers’ Reports

After the demonstration all three specialist-observers (magician, audiovisual expert, and pathologist) concluded, without mutual consultation, that, as Max Maven put it, “Philip Malicdan is a willful fraud” (Maven 1987, personal communication). The three based their conclusions on different evidence and different forms of analysis.

In his report (1987) Maven points out that four different named “elements” of conjuring methodology were present in Malicdan’s work.

1. “Natural law disguised” means that Malicdan seemed to have pushed his hands deeper into a patient’s body than would seem possible.
2. “Secretly prepared materials” refers to the conjuror’s “gimmick,” “gaf,” or “load,” apparently produced from the patient’s body. Malicdan’s “load” consisted of tissue wrapped around blood-clots, a “very efficient [method] in that the *load-container* is the *load-item* itself.” One unused specimen was found in the water-basin in which Malicdan washed his hands before an unexpected hand-search, and traces of another appeared on the bathroom floor after Mrs. Malicdan retreated there to avoid another search.

3. "Secret activities" involves the methods used by Mrs. Malicdan to transfer the "load" to her husband and by Philip Malicdan to conceal it until it could be used.
4. "Psychology" shows Malicdan at his most sophisticated. When he and Frances arrived in Michigan the day before the scheduled demonstration, they were shown the laboratory and approved the general arrangements, the lighting, etc. At that time Malicdan complained of high blood pressure symptoms and insisted on being examined by a physician, who prescribed medication. Maven pointed out that Malicdan used his disability in several ways. It allowed him to quit at any time; in fact, he threatened to do so more than once. It also allowed him to pause "to rest," while he reassessed the situation and figured out how to get another "load." His condition also built sympathy for him and so allowed him to bend or dispense with previously agreed-on procedures. For instance, during one unannounced hand search, he actually held a "load" in his right hand and, in Maven's words, counted on Maven's not choosing "to be so 'cruel' to an overtly suffering man," whereas actually Maven refrained from the examination because insisting on it would, in his opinion, have led Malicdan to refuse to continue.

Maven's conclusion is that Frances Malicdan carried the loads, "which were passed to her husband by way of the water basin or probably at times by other methods." Tom Peterson, the audiovisual technical director, concurred in this conclusion.

Peterson used three videotape cameras to record "an unquestioned record of whatever took place, so that a valid conclusion would be drawn as to whether the alleged phenomenon was authentic or the result of illusion and trickery" and also "to serve as a base [for] scientists planning similar investigations" (Peterson 1986, personal communication). He provided an ideal protocol for the demonstration but noted afterwards that it had been "compromised and altered" by the Malicdans' actions. For example, after agreeing on the lighting arrangements on September 23, the next day Malicdan complained that the lights were too bright. ("The rays are being blocked.") In addition, Frances Malicdan introduced a satchel containing various oils and medications after the original search had been done and produced from it a dark-colored cloth which she and her husband insisted on using and which was, according to Peterson (1986, personal communication), a "perfect color and texture to disguise an object such as was found in the water bowl" (i.e., the "load"). Such tactics, as well as delays during which some cameras were turned off and observers became inattentive, were specifically mentioned by Peterson as opportunities for sleight-of-hand.

In these cases of delay, the overhead "clock reference" camera, which would record in any light and continued to operate during breaks when other cameras were turned off, proved invaluable. From it Peterson was able to note the tactics mentioned above and also "awkward or unnecessary movements," like Malicdan's attempt to readjust the drape on the operating table while holding a styrofoam cup almost empty of the coffee it had held (which he could more naturally have set down). Peterson also noted Malicdan's "sweep" of the surface of the water basin, after which on another camera "we see Frances Malicdan pick up something from the bowl and place it at the small of her back under the

smock.” (Both Malicdans were asked to dress in hospital scrub-suits before the demonstration but were not required to remove their underwear or submit to a strip-search.) When I mentioned a search at one point, Frances Malicdan expressed a need to go to the bathroom, where a blood smear was found on the floor. Peterson also cited the Malicdans’ “singular ability to control the extent and duration of searches”—for example, by announcing that Malicdan “was ready to begin the healing” or by simply redirecting the search to a different location. Peterson’s detailed analysis of the Malicdans’ actions led him to conclude, like Maven, that “the tape record indicates beyond any doubt that sleight-of-hand was used to create the illusion of mysterious withdrawal of diseased tissue from within the patient” (1986, personal communication).

Ullmann provided evidence of a different kind. He was accompanied by his assistant, who acted as one of Malicdan’s patients and also took blood samples to match with the blood and tissue allegedly removed by Malicdan. Ullmann’s report pointed out that, without exception, the samples were in an “advanced state of degeneration,” which “indicates to me that they were not removed at the time of their purported ‘Psychic Surgery’ ” (Ullmann 1986, personal communication). He notes that in two cases where the samples were not too deteriorated to preclude analysis, Malicdan had apparently succeeded in taking Type B blood from patients whose blood type is O.

The tissue specimens, furthermore, showed no relationship to the parts of the various patients’ bodies from which they had allegedly been removed. Ullmann concluded that probably the specimens “were removed—whether from humans or animals I do not know—possibly weeks or months before [the demonstration] and were kept under unsterile conditions to be produced at the appropriate time as fresh specimens” (1986, personal communication). This hypothesis is strengthened by the fact that Frances Malicdan is a medical technologist who would have had opportunities in the Philippines to obtain human blood and tissue. In addition, a friend of Henry Belk’s who supervised the North Carolina treatment of patients told us that a “mysterious box,” which she did not open, appeared in Belk’s refrigerator at the beginning of the Malicdans’ stay and disappeared when they left. But wherever the tissue specimens may have been between the Malicdan’s arrival in the United States and the university demonstrations, Ullmann concurs with Maven and Peterson about the method of delivery: “I . . . assume that these blood clots were collected in the distant past and were kept in a dried state and were mixed with water (in the hand-washing basin) during Mr. Malicdan’s hand-washing procedures immediately prior to the surgery” (Ullmann 1986, personal communication). He added that the seemingly human origin of the blood “found on the surface of the degenerated tissue specimens offers no proof for the human origin of the tissues, since the liquified blood becomes easily attached to the surface of any tissue and gives a positive result” (Ullmann 1986, personal communication).

Discussion

The Reactions of Advocates of Psychic Surgery

The reactions of Henry Belk and some participants in the demonstration are interesting as examples of cognitive dissonance. After receiving Peterson’s report, Belk wrote to him:

I have read and carefully digested your report on Malicdan's alleged healings in Detroit. I have only these complaints. Conditions should have been even tighter. . . . The magician should have been eliminated. . . . Malicdan is a bastard and SOB (who is after money, greed, and Ego). . . . I never rule out FRAUD or its possibility. Yet equally I was healed by [other psychic surgeons], and it's a reputable, daily happening. *Truth will out. All is not fraud.* I am glad you were so thorough. But don't throw out the baby with the water! I admire you and a job well done. [Belk 1987, personal communication]

One of the four subjects from whom Malicdan allegedly extracted blood and tissue during the demonstration was Jewel Irwin Pookrum, M.D., in private practice at a holistic medical clinic in Detroit. Pookrum graduated from the Creighton School of Medicine and was Chief Resident in Gynecology at Henry Ford Hospital, Detroit. Malicdan "operated" on her for adhesions from two caesarean sections. She believes that since "psychic surgeons merge energy fields, theirs and the patient's," and since they "know the natural cleavage points in the cell," they can "enter the body without making an incision" (Pookrum 1986, personal communication). Even though the blood Malicdan allegedly removed or materialized from her was Type B rather than her own Type O, and even though the histologic report showed "completely degenerated and necrotic tissue with necrotic cellular debris in which no other tissue elements can be identified" (Ullmann 1986, personal communication), Pookrum insists today that what Malicdan removed was her adhesions. The fact that all tissues from the four subjects were not only necrotic but similar (though one allegedly came from a lump behind the left knee, another from the back of the neck, a third from an eye, a fourth from the heart region) did not shake her belief. She told me, "This has been just a monumental experience in my life and has truly awakened my horizon on many levels" (Pookrum 1986, personal communication).

The reactions of these believers in psychic surgery are reminiscent of Qaselid and his fellow Kwakiutl shamans, as recorded by Boas (1969[1930]). Though Qaselid provided a detailed description of his shamanic trickery, involving confederates and sleight-of-hand to produce "paranormal" materializations, he and his fellow shamans sustained their belief in the supernatural. After faking the materialization and removing the sickness, the conjuror sang his sacred song: "Do those supernatural ones really see it? Those supernatural ones see it plainly, those supernatural ones. No one can imitate our great friends, the supernatural ones" (Boas 1969[1930]:28). Qaselid learned all the sleight-of-hand tricks of various shamans but still continued to search for a true shaman endowed with true supernatural powers. After much time and searching and after learning many tricks, he believed that perhaps he had found a true shaman: "Only one shaman was seen by me, who sucked at a sick man and I never found out whether he was a real shaman or only made up. Only for this reason I believe that he is a shaman; he does not allow those who are made well to pay him. I truly never once saw him laugh" (Boas 1969[1930]:40-41). Like Qaselid, contemporary advocates of psychic surgery, such as Belk, rest their case on the likely existence of one "true" shaman.

Psychic Surgery: A Minor Philippine Industry

The evidence concerning the alleged phenomenon of psychic surgery points clearly to the conclusion that it is cultural behavior learned by its practitioners,

just as it represents cultural behavior by those patients, Philippine and Western, who accept it as a gift of the spirit. From an anthropological point of view, the history of psychic surgery in the Philippines over the past 20 years presents a clear example of the process of acculturation which has been supported by the ruling oligarchy of the country. Far from being reluctant shamans “deculturated” by Western contact, psychic surgeons have made a vital transition from traditional shamanism (“extraction” from the body of leaves, seeds, worms, hair, etc.) to a simulacrum of Western scientific medicine (“extraction” of blood, tissue, tumors, organs) and from traditional shamanic concepts to Western religious concepts of the Holy Spirit and the saints, Far Eastern concepts of “karma,” “prana,” and “chakras”; parapsychological concepts of “etheric,” “astral,” and “bioplasmic” bodies; or quasi-scientific concepts like the bicameral mind or energy vibrations (Finkelstein 1985:131).

The dramatic transformations of psychic surgeons from barrio and village shamans to global healers is a matter of historical fact and can be dated. The turning point in this movement was the September 1965 article in *Psychic News* by Nelson Decker, a doctor of chiropractic in Los Angeles. Decker described “psychic surgery” and his month-long experience with Eleuterio Terte, the “father of psychic surgery,” and Terte’s leading disciple Tony Agpaoa. In his book on healers and healing, George W. Meek—engineering graduate of the University of Michigan, holder of patents relating to air and thermal control processes, and inventor of “Spiricom” (a device for speaking with the dead)—notes that “The native Filipino healers . . . were totally steeped in nature. . . . then things began to change! *Foreigners* started to show up in search of healing. These foreigners, upon returning home, showed films and extolled the ‘miracles’ of healing” (Meek 1977:100; emphasis in original). Meek acknowledges Henry Belk and Harold Sherman as pioneers in the study of Filipino healers, who laid the groundwork for the researchers who followed them.

By 1973 psychic surgery had achieved de facto acceptance by the Philippine Medical Association (PMA). The PMA concluded that an investigation they conducted had revealed nothing illegal, since psychic surgeons used no instruments and did not write prescriptions. An article in *Philippine Panorama* (August 25, 1973) adds, “Besides, . . . they bring in foreign tourists. These tourists go to these healing centers, and even Americans from Clark Air Base [go] in bus loads.”

In 1983 the Philippine Healers’ Circle, Inc., headed by the psychic surgeon Alex Orbito and with Malicdan as an officer, was inaugurated with a keynote speech by the Philippine government’s Information Minister, who observed that the limits of modern medicine could be transcended by “psychic healing,” which, he noted, includes removing tumors and other objects from patients’ bodies by healers working bare-handed. In 1984 former President Ferdinand Marcos sent a personal message to the First Philippine Conference on Paranormal Healing, organized by the Philippine Healers’ Circle. Marcos, a believer in psychic surgery (Singer and Ankenbrandt 1980:21), said in an address to the Conference (1984), “For over a decade now the Philippines have been the center of world attention for her incredible and admittedly controversial methods of spiritual healing.” He added that “the Philippine government . . . is fully aware of the many benefits that have been reported to have resulted from them” (source not avail-

able). Indeed, by 1984 psychic surgeons were attending international anthropological meetings in Indonesia, South America, and Europe. Though Marcos is gone, political support for psychic surgeons appears to be constant. In 1985 and 1986 two more international conferences on "paranormal healing" were sponsored by psychic surgeons in the Philippines. In short, the existence today of hundreds of psychic surgeons in the Philippines does not indicate any new facts or scientific discoveries about biology or physics but rather clearly supports an anthropological principle that people learn cultural behavior from other people, generally when it is in their interest to do so. With psychic surgery we are dealing with syncretic cultural behavior, the meaning and import of which differs for practitioners and their Western clients and proponents.

NOTES

Acknowledgments. I am indebted to Professor Kate Ankenbrandt for her substantive collaboration as an investigator of psychic surgery. A one-hour edited video of the demonstration is available from Singer-Sharrette Traditional Healing Productions, 588 S. Bates Street, Birmingham, MI 48009. It is titled: "Psychic Surgery: A Case History of Shamanic Sleight-of-Hand."

The author was unable to provide certain journal and page references. Correspondence may be addressed to the author at the School of Health Sciences, Oakland University, Rochester, Michigan 48309-4401.

¹When I visited Malicdan at Baguio in the Philippines in February 1984, a group of 10 Canadians and two young German women were waiting for treatment. The Canadians' three-week package trip was arranged by a Canadian travel service at a cost of \$4,000. This fee did not include treatments by Malicdan at \$50 for the first and \$25 for subsequent sessions. Just after treatment I spoke to a 75-year-old male patient who suffered from chronic bronchitis and sinus trouble and who said he felt much better after his psychic surgery than he did during the preceding six years after medication and surgery by physician specialists. One German woman said she had had diagnosed breast and uterine cancer which Malicdan cured in 1981. Her German physician allegedly told her afterward that she was "all clean." In 1984 she returned to Malicdan for "preventive" treatment.

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